



Incident handler's journal

Date: May 30, 2026	Entry: 1
Description	A small U.S. healthcare clinic was struck by a ransomware attack on a Tuesday morning. Attackers gained initial access through a targeted phishing email containing a malicious attachment. Once an employee downloaded the attachment, malware was installed which then deployed ransomware that encrypted all critical files across the company's systems. Employees were locked out of medical records and essential software, forcing the clinic to shut down operations and contact external organisations for technical assistance.
Tool(s) used	None — this entry documents an initial incident review based on scenario information provided. No technical investigation tools were used at this stage.
The 5 W's	Who caused the incident? An organised group of unethical hackers known to target organisations in the healthcare and transportation industries. What happened? A ransomware attack was deployed via a phishing email. Employees received emails with a malicious attachment; once downloaded, malware installed itself and encrypted all company files. A ransom note was displayed demanding a large payment in exchange for the decryption key. Business operations were completely shut down. When did the incident occur? Tuesday morning at approximately 9:00 a.m. Where did the incident happen? At a small U.S. primary-care health clinic. The attack affected employee workstations across the organisation's network, encrypting files on all connected systems. Why did the incident happen? The attackers were financially motivated and deliberately targeted the healthcare sector. They exploited a lack of employee security awareness — specifically the ability to send phishing emails with

	malicious attachments that employees were not trained to identify and avoid. The absence of email filtering controls and endpoint protection allowed the malware to execute successfully.
Additional notes	Key questions to follow up on: Was the ransom paid? Were backups available to restore from? How many employees received the phishing email and how many clicked? Were other systems on the network compromised beyond the initially infected machine? Priority actions should include isolating affected systems, notifying relevant authorities and regulatory bodies (given the healthcare context and likely patient data involvement), and beginning forensic investigation to determine the full scope of encryption and data exfiltration.

Reflections/Notes: Reflections/Notes: This incident highlights two critical security gaps: insufficient employee security awareness training and the absence of adequate email filtering or endpoint protection. The attack followed the classic phishing-to-ransomware kill chain — a pattern that is well-documented and largely preventable with the right controls. Key lessons: organisations in the healthcare sector are high-value ransomware targets due to the sensitivity of patient data and the pressure to restore operations quickly. This makes them more likely to pay the ransom, which in turn encourages further attacks. Going forward, I want to research how incident response playbooks for ransomware are structured, and what the recommended steps are for containment, eradication, and recovery in healthcare environments where patient safety may be at risk.